Western Cape Government
FOR YOU

New Somerset Hospital

Serving the Community
Department: Occupational Health & Safety

EMERGENCY CARE OF OCCUPATIONAL EXPOSURE TO POTENTIALLY INFECTIOUS BODILY FLUIDS[1]

Staff Injury Report Form

EXPOSED HEALTHCARE WORKER'S DETAILS

Surname .....
Name .....
Folder number .....
DOB .....

Date seen: .....
Time seen: .....
Attending clinician: .....
Date of injury: .....
Time of injury: .....

In which department do you work?

.....

What is your position? (e.g. registrar, student nurse, PN, general worker etc.)

.....

Which institution employs you?
(if you are a registrar – e.g. GSH, RXH, NSH; if you are a student – UCT, UWC, Stell, CPUT etc.)

.....

Type of injury (circle): Sharp / Human bite / Scratch / Splash

Specify item (e.g. needle, blade) and whether it was contaminated with body fluid: .....

How and where did the injury happen? Describe the incident and the injury.

.....

.....

.....

.....

What action did you take after the injury occurred?

.....

.....

Other staff present: .....

SOURCE PATIENT DETAILS

SOURCE CASE STICKER

Surname .....
Name .....
Folder number .....
DOB .....

Ward: .....

[1] Use this in conjunction with the Occupational PEP policy document for NSH.

CONSENT FORM FOR OCCUPATIONAL HEALTH ASSESSMENTS

SECTION A – REASON FOR REFERRAL (please tick as appropriate)

1.	Medical surveillance	☐
2.	Fitness certification	☐
3.	Fitness-to-work / return-to-work fitness assessment	☐
4.	Incapacity management	☐
5.	Occupational injury	☐
6.	Occupational disease (e.g. TB, COVID-19, occupational asthma or dermatitis)	☐
7.	Occupational exposure incident (e.g. needlestick, sharps or splash injury)	☐
8.	Other (please state):	☐

SECTION B – EMPLOYEE'S DETAILS

EMPLOYEE LABEL

If unavailable, complete in writing:

Name & surname: .....

DOB: .....

Contact number: .....

Folder number: .....

SECTION C – EMPLOYEE'S CONSENT

The occupational health service routinely captures data on all persons attending this clinic. The purpose of this data collection is for monitoring and evaluation, quality improvement and statutory reporting. Occasionally, this data may be used for research purposes. All data are kept strictly confidential within the occupational health service. Only relevant data are shared with management and legal authorities; and as far as possible, data are anonymised.

I hereby confirm that the purpose of data collection has been explained to me and I consent to its use for the prescribed purpose. I understand that only in rare circumstances, individual or group information may have to be released without my consent, e.g. if the safety of others may be at risk or if required by law.

Employee signature: .....

HCW signature: .....

Date: .....

CHECKLIST FOR INITIAL ATTENDING CLINICIAN (OHS, EC OR NBC) (tick what you have done)

☐ Check that the exposed area has been thoroughly rinsed / irrigated. ***Assess if infectious / non-infectious.***

☐ IF NON-INFECTIOUS EXPOSURE OR INTACT SKIN:*

- PEP is not indicated, no need for bloods or follow-up at OHS
- Only complete COIDA First Medical Report form and report the incident to the matron

☐ IF INFECTIOUS EXPOSURE:

- Give the HCW the first dose of ARVs and the starter pack (unless he/she is *known* to be HIV infected)
  - Give *within 1–2 hours of exposure*. Definitely < 8 hrs. NO benefit > 72 hrs.
  - Time and date given: .....
    Name + signature: .....
    Available at MATRON's office
    - Starter pack contains a 7-day supply of:
      ☐ Tenofovir 300 mg + Lamivudine 300 mg + Dolutegravir 50 mg as fixed-dose combination, 1 tablet once daily PO
- **Has the HCW been immunised against Hep B** (i.e. received minimum 3 vaccines)? (*circle*)
  - Yes, fully Yes, partially No Unknown
  - If yes – date of last booster vaccine or antibody titre check: .....
  - Assess need for HepB prophylaxis (see **table 2**) and administer vaccine/IG if required.
- Ask about pregnancy/lactation/other medication/conditions which might alter PEP choices
  - Counsel about risks and OHS procedure and give information leaflet or policy document.
  - Advise on the need to practise safe sex for the next 3 months.
- Complete forms:
  - Attending clinician to complete: COIDA First Medical Report + this checklist
  - Staff member/student to complete: Staff injury report form and consent form
- Bloods: clinician/phlebotomist take bloods from the **HCW/student** using **STAFF lab request form A**
  - Ensure that bloods have been done on the **source patient** by the phlebotomist/managing team using **SOURCE lab request form B**.
- Give FOLLOW-UP instructions:
  - Staff members and elective students must report to the OHS nurse the following Monday.
  - Students and contract workers must report to the OHS service at their training institution/employer's health provider within 3–7 days.
- Inform the matron-on-duty of the incident

DECIDING ON PEP INTERVENTIONS

1. HIV prophylaxis	**Status of the source**		
	HIV positive	**Unknown**[1]	**HIV negative**[2]
Intact skin *	No PEP	No PEP	No PEP
Splash or percutaneous injury	**3 drugs**	**3 drugs**	No PEP
Percutaneous injury, mucosal splash or non-intact skin exposure to ***non***-infectious bodily fluids *	No PEP	No PEP	No PEP

[1] If subsequent testing reveals the source to be HIV negative, PEP can be stopped. In the event of the source HIV status remaining unknown, the full 28-day course of PEP should be completed.

[2] Only if confirmed on formal HIV ELISA result & the source patient is **not** high risk on questioning.

Potentially HIV-infectious bodily fluids: Blood, CSF, Semen, Vaginal secretions, Synovial fluid, Pleural fluid, Pericardial fluid, Ascitic fluid, Amniotic fluid, Wound secretions, Breast milk, Human bite

* *Non HIV-infectious bodily fluids:* Sweat, Tears, Saliva (BUT if a human bite wound has punctured the skin, PEP should be given), Sputum, Urine, Stool, Vomitus

2. HEPATITIS B PROPHYLAXIS – CIRCLE BLOCK THAT IS APPLICABLE

	SOURCE CASE RESULT	
HCW	**HepBsAg positive or unknown**	**HepBsAg negative**
Not vaccinated	Do HepBsAg HepB **immunoglobulin**[2] within 72 h* (500 iu or 0.06 ml/kg) and repeat in 1 month *plus* HepB **vaccination**[3] 1st dose with repeat doses at month 1–2–6	HepB vaccination[3] 1st dose with repeat doses at month 1–2–6
Vaccinated with known good vaccine response	Nil	Nil
Partially vaccinated	Do HepBsAg HepB **immunoglobulin**[2] within 72 h* (500 iu or 0.06 ml/kg) *plus* HepB **vaccination**[3] stat then complete vaccine course	HepB vaccination[3] stat then complete vaccine course
Vaccinated with unknown or poor vaccine response (HepBsAb titre < 10)	Do HepBsAg HepB **immunoglobulin**[2] within 72 h* (500 iu or 0.06 ml/kg) and repeat in 1 month *plus* HepB **vaccination**[3] 1st dose with repeat doses at month 1–2–6	HepB vaccination[3] 1st dose with repeat doses at month 1–2–6

	HepB immunoglobulin given				**HepB vaccine given**			
Dose:								
Site:								
Date:								
Given by:								

BLOOD TESTS TO BE DONE ON HCW

Test (see lab form **A** attached)	**Date taken**	**Result**
☐ HIV ELISA		
☐ HepB surface **Antibody** + titre = immunity (if incomplete/unvaccinated also do HepBsAg)		
☐ Hepatitis C antibody		
☐ Creatinine		

BLOOD TESTS[4] TO BE DONE AND INFORMATION NEEDED ON THE SOURCE PATIENT

Test (see lab form **B** attached)	**Date taken**	**Result**
☐ HIV ELISA (if < 18 months do HIV-PCR)		
☐ HepBs**Ag (Antigen)**		
☐ Hepatitis C antibody		
If patient is on HAART:		
☐ Latest HIV viral load		
☐ Current ARVs	AZT D4T TDF ABC 3TC FTC DDI NVP EFV Aluvia Atazanavir DTG	
☐ Previous ARVs	AZT D4T TDF ABC 3TC FTC DDI NVP EFV Aluvia Atazanavir DTG	

[2] Available in Emergency drug cupboard fridge. HBIG 200 iU/2 ml. Adult dose: 500 iU IMI (5 ml). Paediatric dose 0.06 ml/kg IMI.
* Give within 72 hours if HCW's HepBsAg is negative and at risk of possibly contracting Hepatitis B from source.

[3] Available in Emergency drug cupboard fridge. Hepatitis B vaccine 20 mcg/ml. Dose 1 ml IMI.

[4] With informed consent (or superintendent consent as required).

FOLLOW-UP CARE OF OCCUPATIONAL EXPOSURE TO POTENTIALLY INFECTIOUS BODILY FLUIDS

First visit (within 3–7 days) **Date:** .....

- ☐ Containment counselling .....
- ☐ ACTS pre- and post-test counselling .....
- ☐ Results of source patient .....
- ☐ Results of HCW .....
- ☐ Side-effect check .....
- ☐ Drug-interactions check .....
- ☐ Check HepB vaccination schedule .....
- ☐ Encourage safe sexual & work practices .....
- ☐ Offer condoms .....

Follow-up visit (± 2–4 weeks or intercurrent) **Date:** .....

- ☐ Containment counselling .....
- ☐ Side-effect and adherence check .....
- ☐ Give HepB IG & vaccine @ 4 weeks if not immune & source Hep B positive .....
- ☐ ± Repeat creatinine if baseline abnormal .....
- ☐ Repeat FBC if on AZT .....
- ☐ Encourage safe sexual & work practices .....
- ☐ Offer condoms .....

Follow-up visit (6 weeks) **Date:** .....

- ☐ Containment counselling .....
- ☐ Repeat HIV-ELISA .....
- ☐ Repeat creatinine if on TDF .....
- ☐ Repeat FBC if on AZT .....
- ☐ Repeat HepC PCR & Ab if exposed to Hepatitis C .....

Follow-up visit (3 months) **Date:** .....

- ☐ Repeat final HIV-ELISA .....
- ☐ Repeat HepC PCR & Ab if exposed to Hepatitis C .....
- ☐ Encourage safe sexual & work practices .....

Follow-up visit (6 months) **Date:** .....

- ☐ Repeat HepBsAg & HepBsAb titre – if not immune and was at risk of Hep B .....
- ☐ Repeat HepC PCR & Ab if exposed to Hepatitis C .....
- ☐ Encourage safe sexual & work practices .....

NATIONAL HEALTH LABORATORY SERVICE

CLINICAL PATHOLOGY

A - STAFF MEMBER FOR LAB LABELS

PLEASE PRINT IN BLOCKS AND (X) THE APPLICABLE BOXES

MARK IF URGENT ☒

Patient I.D. Number

LOCATION

HOSPITAL / CLINIC: NEW SOMERSET HOSPITAL

Ward: OCCUPATIONAL HEALTH & SAFETY CLINIC

Cost Centre: Z ARV 24

CLINICAL / SPECIMEN DETAILS

Diagnosis / reason for request: A#PEP COIDA needlestick injury: Healthcare worker or student baseline

Medication: Warf Hep

Type of specimen: BLOOD

Taken on: at h

Taken by:

CONTACT DETAILS OF RESPONSIBLE PRACTITIONER

NAME (Prof / Dr / Sr):

Persal or Practice No: 5600634

Cell No: Bleep

Tel(021) 402 6911 Fax (0)

Signature #:

PATIENT INFORMATION

PATIENT LABEL

Patient Hosp./ Clinic Number: Fee class:

Surname:

First name:

Address: Postal Code:

Tel/Cell: Sex M F

D.O.B. (D D M M Y Y Y Y): Age:

PRIVATE PATIENTS ONLY

ICD10 diagnosis codes:

Medical Aid: Plan:

Medical Aid No: Employer:

Authorisation No: Dep code:

Account to / Principal Member:

Member address: Postal Code:

Member Tel. No. (H/Cell): (W):

Member I.D. number:

CHEMICAL PATHOLOGY

General:		Rands
HS* ☐	Blood gases	45
Y ☐	Sodium	25
Y ☐	Potassium	25
Y ☐	Chloride	15
Y ☐	Urea	25
Y ☒	Creatinine	25
Y ☐	Calcium	25
Y ☐	Magnesium	25
Y ☐	Inorganic phosph	25
Y ☐	Uric acid	27
Y ☐	Total protein	25
Y ☐	Albumin	34
Y ☐	Total bilirubin	29
Y ☐	Conj. bilirubin	22
Y ☐	ALP	35
Y ☐	GGT	38
Y ☐	ALT	38
Y ☐	AST	38
Y ☐	LDH	38
Y ☐	Amylase	36
Y ☐	Lipase	36
Cardiac:		
Y ☐	CK	40
Y ☐	CK-MB	38
◷ ☐	Troponin	141
Diabetes / metabolic:		
Grey ☐	Glucose - fast	25
Grey ☐	Glucose - rand	25
P ☐	HbA1c	71
Grey* ☐	Lactate	173
Lipids:		
Y ☐	Triglyceride	56
Y ☐	Total cholesterol	38
Y ☐	HDL-cholesterol	49
Anaemia:		
Y ☐	HDL-cholesterol	59
Y ☐	Transferrin	83
Y ☐	Ferritin	93
Y ☐	Vitamin B12	93
P* ☐	Folate - red cell	131

Endocrinology:		Rands
Y ☐	TSH	146
Y ☐	Free T4	123
Y ☐	Free T3	123
Y ☐	bHCG	87
Y ☐	FSH	93
Y ☐	LH	93
Y ☐	Estradiol	93
Y ☐	Progesterone	96
Y ☐	Prolactin	93
Y ☐	Testosterone	93
Y ☐	SHBG	93
◷ ☐	PTH	127
Y ☐	Cortisol	93
Y ☐	Insulin	93
Y ☐	AFP	93
Cerebrospinal fluid:		
Grey ☐	CSF glucose	25
☐	CSF chloride	18
☐	CSF protein	22

DRUGS

		trough	peak
Y ☐	Paracetamol		80
Y ☐	Salicylate		80
Y ☐	Lithium		39
Y ☐	Methotrexate		80
Y ☐	Tricyclics		80
Y ☐	Phenytoin		80
Y ☐	Phenobarbitone		80
Y ☐	Epilim / Na valpr		80
Y ☐	Carbamezepine		80
Y ☐	Digoxin		92
Y ☐	Theophylline		80
	Urine mandrax		80
P ☐ ☐	Cyclosporin		154
Y ☐ ☐	Amikacin		88
Y ☐ ☐	Gentamycin		88
Y ☐ ☐	Vancomycin		88

Time of dose :

HAEMATOLOGY

General:		Rands
P ☐	FBC	48
P ☐	Differential count	26
P ☐	Film (morph)	26
P ☐	Haemoglobin	15
P ☐	White cell count	15
P ☐	Platelet count	18
P ☐	Reticulocytes	24
P ☐	Malaria screen	43
◷ ☐	ESR	24
Coagulation:		
B ☐	INR	39
B ☐	PTT	44
B ☐	Fibrinogen	28
B ☐	D-dimers	216
B ☐	Thrombin time	56
B ☐	Antithrombin III	173
B ☐	Lupus anticoag	197
B ☐	Protein C	238
B ☐	Protein S	293
B ☐	APC Resist	205
Other:		
P ☐	Coombs	29
© ☐	CSF/Fld cytosp	52

VIROLOGY

HIV testing:		Rands
Y ☒	HIV serology	97
PP ☐	HIV viral load	318
P ☐	PCR : HIV	387

Hepatitis serology:

Y Clinical hepatitis: 105 ea ☐ A ☐ B ☒ C

Y Hepatitis immunity: ☐ A ☒ B

Other serology:	IgM 102	IgG 94
Y CMV	☐	☐
Y EBV	☐	☐
Y HSV	☐	☐
Y VZV	☐	☐
Y Rubella	☐	☐
Measles	☐	☐
Y Mumps	☐	☐

Viral isolation:		
P ☐	CMV pp65 Ag	323
☐	Rapid RSV	87 ea
☐	Rap. rota/adeno	169
☐	Culture (specify)	87
☐	PCR (specify)	var

IMMUNOLOGY

P ☐	CD4 (PLG)	64
Y ☐	CRP	60
Y ☐	IgG, IgA, IgM	55 ea
Y ☐	Total compl	97
Y ☐	Syphilis Serology	16
Y ☐	ASOT / DNase B	87
Y ☐	Toxoplasma	94
Y ☐	ANF	116
Y ☐	Anti ds-DNA	116
Y ☐	Anti cardiolipin	93
Y ☐	ANCA	232
Y ☐	RF	61
Y ☐	ENA	115 ea

MICROBIOLOGY

Specimen site:

General:		
Y ☐	Routine MC&S	≥81
Y ☐	CSF cell count, microscopy & culture	≥141
Y ☐	Fungal M&C	≥67
Y ☐	Blood culture	≥101
Y ☐	Parasites	≥57
TB investigation:		
☐	Microscopy	32
☐	Culture	≥101
☐	Sensitivity	≥190
Antigen detection:		
☐	Cryptococcal Ag	40
☐	Pneumocystis	87
☐	Chlamydia	116
☐	C. difficile toxin	217

OTHER TESTS (ALL DISCIPLINES)

For full repertoire of tests, consult laboratory handbook

1. HIV-ELISA
2. Hepatitis B surface antibody titre
3. Hepatitis C antibody
4. Creatinine

(2 x yellow tubes)

RECEIVED IN LAB

PROVINCIAL REQUIREMENTS :

Specimens may be rejected if Patient name and number, Practitioner name, signature and Persal / Practice number, or Hospital / clinic / ward are omitted.

Signature & PERSAL No. of Consultant:

I hereby certify that I have considered the repertoire of tests requested as well as previous test requests for this patient and confirm that this form is duly completed and will not result in any fruitless and wasteful expenditure.

SORT	LABEL

NATIONAL HEALTH LABORATORY SERVICE

CLINICAL PATHOLOGY

B - SOURCE CASE FOR LAB LABELS

PLEASE PRINT IN BLOCKS AND (X) THE APPLICABLE BOXES

MARK IF URGENT ☒

Patient I.D. Number

LOCATION

HOSPITAL / CLINIC: NEW SOMERSET HOSPITAL

Ward: OCCUPATIONAL HEALTH & SAFETY CLINIC

Cost Centre: Z ARV 24

CLINICAL / SPECIMEN DETAILS

Diagnosis / reason for request: A#PEP SOURCE CASE in an occupational exposure incident (COIDA)

Medication: Warf Hep

Type of specimen: BLOOD

Taken on: at h

Taken by:

CONTACT DETAILS OF RESPONSIBLE PRACTITIONER

NAME (Prof / Dr / Sr):

Persal or Practice No: 5600634

Cell No: Bleep

Tel(021) 402 6911 Fax (0)

Signature #:

PATIENT INFORMATION

Patient Hosp./ Clinic Number: Fee class:

Surname:

First name:

Address: PATIENT LABEL

Postal Code:

Tel/Cell: Sex M F

D.O.B. (D D M M Y Y Y Y): Age:

PRIVATE PATIENTS ONLY

ICD10 diagnosis codes:

Medical Aid: Plan:

Medical Aid No: Employer:

Authorisation No: Dep code:

Account to / Principal Member:

Member address: Postal Code

Member Tel. No. (H/Cell): (W):

Member I.D. number:

CHEMICAL PATHOLOGY

General:		Rands
HS* ☐	Blood gases	45
Y ☐	Sodium	25
Y ☐	Potassium	25
Y ☐	Chloride	15
Y ☐	Urea	25
Y ☐	Creatinine	25
Y ☐	Calcium	25
Y ☐	Magnesium	25
Y ☐	Inorganic phosph	25
Y ☐	Uric acid	27
Y ☐	Total protein	25
Y ☐	Albumin	34
Y ☐	Total bilirubin	29
Y ☐	Conj. bilirubin	22
Y ☐	ALP	35
Y ☐	GGT	38
Y ☐	ALT	38
Y ☐	AST	38
Y ☐	LDH	38
Y ☐	Amylase	36
Y ☐	Lipase	36
Cardiac:		
Y ☐	CK	40
Y ☐	CK-MB	38
☐	Troponin	141
Diabetes / metabolic:		
Grey ☐	Glucose - fast	25
Grey ☐	Glucose - rand	25
P ☐	HbA1c	71
Grey* ☐	Lactate	173
Lipids:		
Y ☐	Triglyceride	56
Y ☐	Total cholesterol	38
Y ☐	HDL-cholesterol	49
Anaemia:		
Y ☐	HDL-cholesterol	59
Y ☐	Transferrin	83
Y ☐	Ferritin	93
Y ☐	Vitamin B12	93
P* ☐	Folate - red cell	131

Endocrinology:		Rands
Y ☐	TSH	146
Y ☐	Free T4	123
Y ☐	Free T3	123
Y ☐	bHCG	87
Y ☐	FSH	93
Y ☐	LH	93
Y ☐	Estradiol	93
Y ☐	Progesterone	96
Y ☐	Prolactin	93
Y ☐	Testosterone	93
Y ☐	SHBG	93
☐	PTH	127
Y ☐	Cortisol	93
Y ☐	Insulin	93
Y ☐	AFP	93
Cerebrospinal fluid:		
Grey ☐	CSF glucose	25
☐	CSF chloride	18
☐	CSF protein	22

DRUGS

Y ☐	Paracetamol	80
Y ☐	Salicylate	80
Y ☐	Lithium	39
Y ☐	Methotrexate	80
Y ☐	Tricyclics	80
Y ☐	Phenytoin	80
Y ☐	Phenobarbitone	80
Y ☐	Epilim / Na valpr	80
Y ☐	Carbamezepine	80
Y ☐	Digoxin	92
Y ☐	Theophylline	80
	Urine mandrax	80
		trough peak
P Cyclosporin	☐ ☐	154
Y Amikacin	☐ ☐	88
Y Gentamycin	☐ ☐	88
Y Vancomycin	☐ ☐	88

Time of dose :

HAEMATOLOGY

General:		Rands
P ☐	FBC	48
P ☐	Differential count	26
P ☐	Film (morph)	26
P ☐	Haemoglobin	15
P ☐	White cell count	15
P ☐	Platelet count	18
P ☐	Reticulocytes	24
P ☐	Malaria screen	43
☐	ESR	24
Coagulation:		
B ☐	INR	39
B ☐	PTT	44
B ☐	Fibrinogen	28
B ☐	D-dimers	216
B ☐	Thrombin time	56
B ☐	Antithrombin III	173
B ☐	Lupus anticoag	197
B ☐	Protein C	238
B ☐	Protein S	293
B ☐	APC Resist	205
Other:		
P ☐	Coombs	29
© ☐	CSF/Fld cytosp	52

VIROLOGY

HIV testing:		Rands
Y ☒	HIV serology	97
PP ☐	HIV viral load	318
P ☐	PCR : HIV	387
Hepatitis serology:		
Y	Clinical hepatitis:	105 ea
☐ A ☒ B ☒ C		
Y	Hepatitis immunity:	
☐ A ☐ B		

Other serology:	IgM	IgG
	102	94
Y CMV	☐	☐
Y EBV	☐	☐
Y HSV	☐	☐
Y VZV	☐	☐
Y Rubella	☐	☐
Measles	☐	☐
Y Mumps	☐	☐

Viral isolation:		
P ☐	CMV pp65 Ag	323
☐	Rapid RSV	87 ea
☐	Rap. rota/adeno	169
☐	Culture (specify)	87
☐	PCR (specify)	var

IMMUNOLOGY

P ☐	CD4 (PLG)	64
Y ☐	CRP	60
Y ☐	IgG, IgA, IgM	55 ea
Y ☐	Total compl	97
Y ☒	Syphilis Serology	16
Y ☐	ASOT / DNase B	87
Y ☐	Toxoplasma	94
Y ☐	ANF	116
Y ☐	Anti ds-DNA	116
Y ☐	Anti cardiolipin	93
Y ☐	ANCA	232
Y ☐	RF	61
Y ☐	ENA	115 ea

MICROBIOLOGY

Specimen site:

General:		
Y ☐	Routine MC&S	≥81
Y ☐	CSF cell count, microscopy & culture	≥141
Y ☐	Fungal M&C	≥67
Y ☐	Blood culture	≥101
Y ☐	Parasites	≥57
TB investigation:		
☐	Microscopy	32
☐	Culture	≥101
☐	Sensitivity	≥190
Antigen detection:		
☐	Cryptococcal Ag	40
☐	Pneumocystis	87
☐	Chlamydia	116
☐	C. difficile toxin	217

OTHER TESTS (ALL DISCIPLINES)

For full repertoire of tests, consult laboratory handbook

1. HIV-ELISA
2. Hepatitis B surface antigen
3. Hepatitis C antibody
4. Syphilis serology

(2 x yellow tubes)

(if < 18 months old do HIV-PCR – purple tube)

RECEIVED IN LAB

PROVINCIAL REQUIREMENTS :

Specimens may be rejected if Patient name and number, Practitioner name, signature and Persal / Practice number, or Hospital / clinic / ward are omitted.

Signature & PERSAL No. of Consultant:

I hereby certify that I have considered the repertoire of tests requested as well as previous test requests for this patient and confirm that this form is duly completed and will not result in any fruitless and wasteful expenditure.

SORT	LABEL

labour

Department:
Labour
REPUBLIC OF SOUTH AFRICA

Claim Number: .....

FIRST MEDICAL REPORT IN RESPECT OF AN ACCIDENT

COMPENSATION FOR OCCUPATIONAL INJURIES AND DISEASES ACT, 1993 (Act No. 130 OF 1993)

[Section 6A(b) – Commissioner's rules, forms and particulars – Annexure 15]

Names and Surname of employee .....

Identity Number ..... Address: .....

..... Postal Code .....

Name of employer .....

Address .....

..... Postal Code .....

Date of accident .....

1. Date of your first consultation .....
2. How did the alleged accident happen? .....

   .....
3. Full clinical description of injury (ies) **(not symptoms, signs or syndromes)** .....

   .....

   .....

   .....
4. Describe briefly any *pre-existing* defect disease .....

   .....

   .....
5. X-rays Date ..... By whom .....
   (Attach report if available)
6. Surgical Procedures: Date ..... By whom .....

   Brief description .....

   .....
7. Anaesthetics: General / Local Duration .....
8. (a) Consultation Yes / No With whom ..... Date .....

   (b) Was the employee referred for physiotherapy? Yes / No Physiotherapist .....
9. (a) Is the employee unfit for work? Yes / No .....

   (b) Possible date fit for: Light duty ..... Normal duty .....

I certify that I have by examination, satisfied myself that the injury(ies) of the employee is the result of the accident as described above.

Signature of Medical Practitioner/Chiropractor .....

Name (Printed) ..... Date (important) .....

Address New Somerset Hospital, Portswood Road, Green Point, Cape Town

..... Postal Code 8001 Practice number 5600634

N.B.: This report must be handed to the injured employee or sent to the employer within 14 days from the date of first consultation.